

WEEK 11 · CAREGIVING

CLASS 2

Counting Care, Seeing Care

READING 1

Excerpts from Chapters 3 and 4, and “Chapter 6: Children and Global Health” in *Children As Caregivers* – Hunleth

Children As Caregivers: The Global Fight Against Tuberculosis and HIV in Zambia | Hunleth

Week 11, Class 2

Chapter 3: Between Silence and Disclosure — pp. 86–99

Chapter 4: Following the Medicine — pp. 101–103; 107–124

Chapter 6: Children and Global Health — pp. 145–155

Hunleth, Jean. 2017. "Chapter 3: Between Silence and Disclosure (pp. 86–99 only)." "Chapter 4: Following the Medicine (pp. 101–103 and 107–124 only)." and "Chapter 6: Children and Global Health." In *Children As Caregivers: The Global Fight Against Tuberculosis and HIV in Zambia*. Rutgers Series in Childhood Studies. Rutgers University Press. pp. 86–99; 101–103; 107–124; 145–155.

CONTEXT FOR READING

Chapter 3's opening pages are omitted from this reading. Here's what you need to know...

This chapter opens with Mr. Simwonde, a father who joins Hunleth's study on one condition: that his son never learn his diagnosis ("We say it is malaria"). Almost no guardians in George told children about TB, and none disclosed HIV. A vast public health literature reads such silence as stigma and denial, a failure to be corrected through counseling. Hunleth writes against that judgment. Older residents remember a time when TB was treatable and "we used to care," and they sort the disease into "TB of the past" and "new TB," the kind that signals HIV and shame. Within this history, adults cast children as innocent and at risk and themselves as potential transmitters, so that concealing a diagnosis becomes something a good parent does. You'll pick up here, with what adults' silences mean and what children know, perform, and conceal in turn.

foods to meet their nutritional needs. The foods given to TB patients tempted children because they were luxuries not eaten on a daily basis, such as fruits, meats, eggs, and juices.

Adults constructed themselves as actors and children as victims within the epidemic. “Careless” adults were blamed for giving TB to children. Children were at risk of getting TB “because they can get it from an adult who has TB.”¹⁸ With children’s innocence as a persistent theme in commentaries about TB, the guilt and responsibility of adults who did not protect children from TB was highlighted. Such a construction has placed high demands on adults—particularly women—to forestall the spread of the disease and protect children.

The presumption of guilt and the great responsibility to protect children was particularly worrisome for TB sufferers I knew. As one woman suffering from TB pointed out, “it is not good to spread the disease to children.” She and others worried not just about social blame, but about the very real suffering that TB might cause the child and the strain it would place on household resources. Sick adults were put in a difficult position in which they had to ensure that TB was not spread to children and yet they had few resources at hand to do so. These evaluations and material challenges shaped how they chose to reveal or conceal their disease.

ADULTS: REVEALING AND CONCEALING

The evaluations within popular histories and taxonomies of TB placed adults in precarious positions as they navigated a complex moral terrain where they were alternately feared, stigmatized, viewed as careless, and called upon to act responsibly. TB sufferers and other adult caregivers expressed a sense of urgency to ensure children’s safety. This urgency included managing children’s fears and providing them with material and emotional supports, and these included managing children’s knowledge of TB. It was, in fact, often impossible for me to disentangle social constructions of TB disease and TB spread from ideas about disclosure to children. Take for example the connection one woman made between concealing knowledge of TB from children and protecting them from disease: “If the children are fifteen years and above, I can tell them that I have TB. If the children are young, I can make sure I prevent them from getting TB.” Her linking of TB disclosure to age makes two ideas evident: protection is tied to knowledge and younger children need protection rather than knowledge.

In the introduction to this chapter, I described my conversations with Mr. Simwonde and other guardians who did not wish to mention TB to the children in their households. Instead of seeing disclosure as protective, they identified concealment as a form of protection and spoke of the illness through other terms. Some described it as malaria. Others called it *matenda*, a Nyanja word

that translates as sickness or disease. Still other families referred to illness as *mendo*, or legs, a term used during a sickness when a person's legs become weak or they can no longer walk.

An exception to this concealment was when a guardian could completely divorce their TB from HIV, which became especially clear in one mother's explanation for how and why she told her daughters Abby and Chiko about her TB. The mother, Elesia, knew that people were skeptical of statements that deny HIV when you have TB. So as proof that she did not have HIV, Elesia invoked her family history of TB, referring to her disease as "family TB" and "TB of the past." She also wore a yellow wristband as additional proof. The wristband was one of many given out after a person received an HIV test from a New Start HIV Counseling and Testing Centre. New Start Centres are directly managed or franchised by Population Services International (PSI), a nongovernmental organization that is based in Washington, DC, and works in Zambia and other countries. Written on the wristbands was the question: "I know, Do you?" Wristbands were given to everyone who received a test, regardless of the result. They served as a form of branded advertising meant to, in the words used to describe the intervention on PSI's website, "help create awareness and generate demand for HIV Counseling and Testing."

The wristbands are a means of public disclosure—disclosure that one has tested for HIV. Elesia had never been to a New Start Centre and had borrowed the yellow wristband from a friend. She told me that people in George know that you do not have HIV when you wear the wristband. This was, of course, not the type of message the developers of the campaign had in mind. Nevertheless, the wristband was a highly visible cue indicating that Elesia did not have HIV. It, along with other cues, freed Elesia to speak of her TB diagnosis with her daughters. When I asked if they knew about her TB, she said: "I told them 'Do not worry, I will recover because it is not AIDS.'" Elesia's actions and statements suggest that HIV, rather than TB, is a primary, underlying concern when considering TB disclosure. She also demonstrated the lengths to which a person must go to prove HIV seronegativity when HIV is the default expectation.

With the exception of Elesia, most guardians in my study did not name TB when talking to the children in their households or extended families. They gave two main reasons, each of which carried a number of implied concerns, including the link between HIV and TB and the association of TB with moral transgressions, suffering, and death. They worried that children might reveal information about TB in inappropriate ways. Encapsulated in this concern was the idea that children may use the term TB haphazardly or disclose a family's private information at school or in their neighborhood.

The idea that children do not keep information private shaped the advice patients and families received from clinic workers and volunteers at George

Health Centre, where most people in George received TB treatment. Explaining why they advised people not to tell their young children, one longtime worker told me that, if children are told, “They’ll be saying, ‘Eh, my relative is suffering from TB.’ You know children, they don’t hide.” Children’s explicit naming of TB was viewed as potentially harmful to the TB patient, the child, and the family. The reasons why people did not want other people to know or openly verbalize TB were frequently related to issues of care. Explicit naming of TB was seen as harmful to a sick person’s well-being because of TB’s association with death. Adult caregivers worried that a child may reference TB in ways that might cause the patient to give up or make them think about their prognosis. They also worried that children would sever their own relationships unknowingly by sharing news of a guardian’s diagnosis. For example, Mr. Simwonde explained: “If I told him, he could have explained the news [to others, especially to his friends]. He can simply be disappointed from his friend. From his side, he cannot feel right.”

The second reason that adults gave for not telling children was the belief that disease knowledge had an effect on a child’s well-being. They worried that their children might reject them or be disappointed in them. They suggested that a guardian’s TB or HIV status could prompt children to think too much [*kuganiza maningi*] and, when people start to “think too much,” they might easily lose hope and feel isolated. In this view, knowledge has an effect on people, and specific knowledge has a negative effect on children. When I asked Mr. Simwonde to explain the process of thinking too much as it relates to disclosure of TB to his son, he told me in English: “He is too young. He can have [an] inferior complex—this brings life expectancy of a human to be little. It cannot build confidence [in him].” Mr. Simwonde’s statement attests to the ways in which illness can reduce a person to mere existence. However, it is not the person who is sick that he refers to. He recounts, instead, the ways in which parental illness can reduce a child’s life.

Unlike in much of the HIV disclosure literature, stigma and denial were not the main analytical categories that drove nondisclosure in my study. Instead, what drove nondisclosure was a guardian’s effort to retain relationships and normative roles and identities under highly uncertain conditions. Rather than constituting bad parenting, not telling was an effort to be a good parent.

CHILDREN: KNOWING AND PERFORMING

I was faced with a dilemma when I began working with the children. I wanted to respect adults’ views of protection and risk, but I also did not wish to mislead the children. I decided to approach the situation by not bringing up any disease names during the study unless the children spoke them first. While this approach was much more in line with local modes of talk about sickness that I described above, in which disease names were de-emphasized or circumvented

in place of other expressions, it yielded little information about TB specifically. Several months after I began the research, I had yet to hear any of the children use the terms TB or HIV in my presence or the guardians use those terms in conversation when their children were present. Whenever I asked the children what their family members were suffering from, they told me “legs,” “malaria,” or just “disease.” Even Abby, Elesia’s daughter, did not speak out loud about TB.

About five months after I met many of the children, I asked them to participate in what we called “children’s workshops,” as a way to for me to gather different insights on children’s disease knowledge and experiences of household illness. The children’s workshops took place in a freestanding building, known as the Demonstration Room, which sat on the grounds of George Health Centre. The children came for the children’s workshops in groups of approximately ten children during three separate afternoons. We played games and had group discussions, and the children created role plays.

The novelty of the children’s workshops was the purposeful exclusion of adults, an aspect that thrilled the children and met with approval from adults, if not a little teasing about when they should come for an adults’ workshop. The theory behind the workshops was that knowledge is always situated. That is, the settings of research and the people present shape the ways in which children act and respond to research questions (Edwards and Alldred 1999). Group discussions offer a valuable addition to household ethnography because questions can be framed in more general terms (Morgan et al. 2002), whereas questions about illness in the presence of a sick family member might always feel personalized, even when phrased in general terms.

During the children’s workshops, I provided a rough framework of activities that focused on illness generally, rather than TB or HIV specifically. I wanted the sessions to be guided by children’s illness concerns and not by biomedical categories and labels that may or may not hold relevance to their daily lives. And because I was gathering a range of children who did not previously know one another, I wanted the research to be sensitive to each individual child’s suffering and circumstances. Twelve-year-old Musa, for example, had watched his older brother die two months before the workshop. Ten-year-old Sarafina’s mother was again bedridden, her condition worsening. Ten-year-old Rhoda’s “small mother,” her mother’s sister, was also bedridden. And the sheer number of TB sufferers in eight-year-old Gift’s household—his grandmother, grandfather, sister, and uncle—infused daily life with uncertainty.

“LIVING IN A HOUSE WITH A SICK PERSON”

Musa, Sarafina, Rhoda, and Gift formed one of ten groups across three workshops that acted out a role play on the theme: “living in a house with a sick person.” The

theme was the only direction I gave the children, and I avoided terms such as care, so as not to influence the direction of the role plays. I had yet to hear any of the children use the terms TB or HIV. During the months prior to the workshop, Musa had told me that his brother was suffering from *mendo* [legs]. Sarafina said that her mother had malaria, and Gift and Rhoda talked about their family members' illnesses as *matenda* [sickness].

In the role play I am about to describe, Musa played the doctor, Gift played a sick man, and Rhoda and Sarafina played Gift's aunt and mother. The group started with the sick man arriving at a clinic with his mother and aunt. The doctor examined the sick man, exclaiming: "He! He's caught TB. I am going to give you this medicine. The way you're supposed to take this medicine. At night, you drink two. And again in the morning at 6, you take two. And again, in the daytime, you take two after eating. Do you understand?" The doctor then handed an imaginary bag of pills to the sick man's aunt, saying: "If it doesn't work, you come back." The three children playing the sick man, mother, and aunt walked slowly to the other side of the Demonstration Room, representing a long distance between clinic and home. Once home, the sick man lay on a bench, with his mother and aunt watching over him. The mother and aunt gave the man medicine, watched him, fed him, told him to sleep, and woke him up to take more medicine. After a couple of quiet moments passed, the aunt decided: "The medicine is too big and it is not working."

The family set out for the clinic and, when they arrived, the doctor told the man: "It seems like your TB is too strong." He then reexamined him, patting his arm and talking in a low voice. He listened to the man's lungs and offered a new diagnosis: "*Imwe, muli na double* [You, you've got double]." To clarify his euphemism, he repeated the diagnosis: "*Muli na HIV na TB* [you have HIV and TB]." With this news, the doctor explained that the man needed further medicine and injections. After giving him an injection, the family returned to their fictive home across the Demonstration Room, never mentioning the words HIV or TB. They altered their daily routines to remind the man to take his medicine. They encouraged him to sleep and eat. And they spent long intervals silently watching over him. Eleven minutes after the role play began, the children ended it. The man's illness had yet to be cured and he remained sick, lying on the bench until the audience applauded.

Across the ten role plays conducted by different children during three separate workshops, all but one role play focused on detailed accounts of TB treatment. Even Mulenga, Mr. Simwonde's son, played a doctor treating a patient who had a combination of what he diagnosed as "TB and sugar disease." Their TB diagnoses were not, however, definitive diagnoses. In each role play, the sicknesses seemed to get worse and involve numerous trips to the clinic. New diagnoses and treatments layered onto old ones, and there remained much uncertainty about diagnosis, treatment, and prognosis.

PRESENCE IN ADVICE AND ISOLATION

During the children's workshops, I asked the children to develop lists of the sicknesses that most concerned them. On their lists, the children included a range of biomedical and local terms for diseases, as well as certain things that observers might not consider to be diseases, such as ARVs.¹⁹ They listed TB in all three workshops. After they developed the list, Olivious went through each item, asking a similar set of questions about each. When she reached TB, the children were quiet. In one group, she finally asked: "Have any of you talked about TB with your family?" To this question, one girl said: "My grandfather, mother, and my sister suffered from TB and took medication." The phrasing of the girl's answer prompted Olivious to also rephrase her question: "How do you know when someone has TB?" This was a question on which the children had much to say.

Children pointed to learning about TB from adults through restrictions and changes in the household that were intended to avoid transmission. Agnes replied: "I was told by my grandmother when they came back from the clinic, 'your elder sister is suffering from TB, so you should not use the same cups and plates with her.'" As an aside, the adults in Agnes's household emphasized several times that they did not disclose Rebecca's disease label until well into her treatment, and only in response to Agnes's persistent questioning about the medicines that Rebecca was taking. Elesia's daughter, Abby, replied to our question: "When my mother came back from the clinic she was using her own utensils and we were told not to eat left out food by her or use her face towel." Several girls spoke of not being able to sleep in the same bed as mothers or aunts, an aspect that was so upsetting to Abby's sister, Chiko, that, when using my tape recorder, she suggested that she and Abby should also get sick. This way, she reasoned, they could again sleep with their mother.

Children associate diagnoses with their own and the sick person's sanctioned and unsanctioned behavior. Only a few years prior to my study, the standard clinical advice given to people who suffered from TB was isolation. To avoid transmission, TB patients and their families were told to eat and sleep separately. During my research, nurses and TB treatment supporters—volunteers who assisted at the health center and provided community outreach—often talked about this advice as outdated and stigmatizing.²⁰ Once on treatment for two or more weeks, TB sufferers are usually not considered to be infectious, and much more transmission occurs prior to diagnosis. For this reason, health practitioners stressed the importance of not abandoning those who suffer from TB. When I asked a volunteer who gave support to people who were on TB treatment about the advice he gave when someone first received their diagnosis, he told me:

First on the eating system: You see, many people are dying with TB because after being diagnosed with the TB, they were being isolated. That was depressing other people's minds [the minds of those with TB] so they die of depression. This was not good. So we introduced using separate plates for relish. If it's rice, then each one has his separate plate. In the environment, especially where that person sleeps should be very clean. In the morning he should bathe. The windows should be opened. Children should be isolated in the other room alone.

The items he listed corresponded with the range of changes that children noted in their discussions of how they knew someone had TB. However, despite his overt focus on how to remain close without transmitting TB, his statement demonstrates the continued concern over children's proximity to TB sufferers. Children received both messages—about avoidance and also about remaining close as good care.

PRESENCE IN CHILDREN'S CAREGIVING

During the household research, I asked each child to make drawings of "taking care of a sick person." These drawings were designed as a way for children to illustrate their ideas about care and a tool for me to use to engage them in conversation about caregiving. Ten-year-old Annie's drawing offers one such example. Annie lived in a one-room rental unit with her mother, Enelesi, and her "sister," the daughter of Annie's oldest sister, who was deceased. In Annie's drawing of "taking care of a sick person," she showed just how sick Enelesi had become and many aspects of how she viewed her role in giving care. Though Enelesi was adamant that she had not disclosed her TB or HIV diagnoses to Annie, the presence of TB was clear in her drawing, as it was in the other children's drawings.

Annie depicted a set of four scenes on a white sheet of paper. At the top, she showed her mom waiting in bed. In her words, "She wants to vomit so I hold her hand to take her outside." In the middle, she depicted herself giving her mother her daily medicine with a glass of drinking water. Next to that scene, she drew herself "chatting and eating" with her mother. Annie showed how she and her mother used clinical advice of eating with separate bowls to continue to have their meals together, making tangible and reinforcing their connection to each other.²¹ Other children showed themselves involved in bringing special foods to their sick guardians, particularly fruits, which are luxury items in George and a symbol of love when given to the sick. The final picture on the page was Annie accompanying her mother to George Health Centre to collect medicine, something that Annie did not in fact do, but these visits were a part of her mother's weekly routine to collect TB medications and ARVs. TB and HIV are present in all four drawings. Even though Annie's mother did not wish to tell Annie about



Fig. 3.1 Drawing of Irene taking care of a sick person. She gives the sick person an orange and drinking water. Drawing by Irene, age twelve.

her TB or HIV diagnoses, Annie was involved in her mother's care, a process that gave her a detailed knowledge of the diseases.

Special foods and medicines were integral actors in child-guardian interactions (see fig. 3.1). They gave children and adults a way to address illness and suffering as well as nurture each other without directly naming a disease. Take, for example, a story that eleven-year-old Rose recorded in February 2008, seven months after her grandmother had begun TB treatment. I had given her a series of pictures to view as she told stories on my tape recorder. To tell her stories, she hid in an empty room in her family's home, a common tactic children employed when using the recorders. The children rarely wanted family members to listen to what they had to say. Speaking on a picture of a person lying in bed, surrounded by family members, Rose said:

This is my father. He is suffering from TB, and he is telling his children: "I am sick my children. But my children, I am taking my medicine. I went to the hospital and I was given this medicine. In case I might forget my dosage, you should remind me. And they have also given the vitamins. These are the TB tablets. I was also told to eat a lot of food and fruits." He later got better because he followed his medication very well. My father didn't die. I love you, Dad.

In Rose's story, her fictive father does not name TB so much as make evident his bodily needs, manage their fears, and give them caregiving roles. There is a subtle but important distinction between this form of making a disease known and the more Euro-American assumption about disclosing as the act of revealing a name to a sickness or condition. His making known—or disclosure—was about caring actions that occurred between people. It was about relationships and responsibilities to one another, rather than indicative of the ends of them. In Rose's words, "My father didn't die."

CONCEALMENT AS EMOTION WORK

Returning to the children's role plays on living with a sick person, a consistent theme across the role plays was the explicit naming of TB by the children who played the doctors but no other children in the groups. The role plays seemed to make a clear point: just as adults avoid saying TB, children also engage in tactical avoidances and concealments. In their strategic usage and avoidance of naming TB in their role plays, as well as in their daily encounters in their homes, children demonstrated an understanding that naming disease has effects on others and on their relationships, something that is hardly different from my observations about how adults approached naming with children.

I have documented only one case in which two girls named TB and HIV in front of their sick uncle. Mateo, their uncle, had moved to their house from

nearby when he could no longer care for himself. After he moved in, the children reportedly used the words TB and HIV directly in front of their uncle, saying both diagnoses loudly when talking with neighbors and friends about Mateo. Family members viewed their direct naming to and near Mateo as a rupture in care for the sick man. During several of my visits, Mateo's brother, the head of the household and father of one girl and an uncle of the other, worried aloud to me. He believed that their direct naming might have harmful effects on Mateo, causing him to "think too much" about his own future and lose hope for recovery. And even though the girls carried out much of the work of caring for the man, such as bringing him food and preparing his bathwater, their work was never acknowledged as "care." When I asked Mateo and other household members if they might consider the girls' activities as care, they spoke instead of the girls' naming, avoidances, and the negative sentiments they expressed when giving him food, preparing his bathwater, and performing other chores for him. Recall earlier in the chapter that this was the type of behavior that adults feared might happen.

The children could not have been aware of Mateo's HIV seropositivity because even he did not know at the time. His HIV tests were negative. Only a test at the very end of my study showed that he was, in fact, positive. I cannot be sure why the children disliked their uncle so much or of the history of their relationship with him. What I do know is this: since his arrival, the burden of care for him had been placed on them and caring for him subsumed household resources and took the girls away from their studies and other activities.

In all but this one household, not naming, revealing, or confirming seemed to serve as a critical part of children's attempts to give care to parents and guardians. Rather than something that prevented care and relationship building, concealment served as a mode of care and an effort to negotiate belonging at a time when their proximity to the sick person and the household was in threat. We might consider concealment, then, as just one aspect of children's range of practices to nurture the sick, such as reminding guardians to take their medicine, making food and preparing bathwater, cleaning the house, attending school without complaint, or—in two brothers' case—lying to their mother about going to school when they were out making money to support the household. It was important to many children to sit with the sick, make them laugh, and encourage them that they would get better. This demonstrates the children's keen awareness of the need to manage emotions, what anthropologist Arlie Hochschild has called "emotion work" (Hochschild 2003). We should not underestimate the importance of children's emotion work in households dealing with illness or the amount of effort children put into managing their own and others emotions.

Children's emotion work related to their worries that relatives might give up, a process that could lead to death. In one of eleven-year-old Paul Kangwa's

recordings, for example, he made clear how pain and suffering from TB might diminish a person's desire to live, inducing them to give up on life. Speaking in the voice of a sick father, Paul said: "Let me die. It's okay even if I die." In the recorded conversation Paul enacted, he worked to persuade his father that he would get better. By following the rules around talk about particular diseases, children showed themselves to be responsive to the needs of relatives. There were significant rewards for such behavior. It ensured their continued presence in households and proximity to relatives; it helped them cultivate and reaffirm their relationships with their relatives; and it helped them stay alive.

Seeing children's concealments as emotion work demands that we recognize the lengths to which children may go to nurture the sick. While much of children's emotion work went unnoticed by adults in George, as it does in many places in the world, Annie's mother, Enelesi, described the value of Annie's efforts. Recall Annie's four scenes of caring for her mother. During the time Annie was drawing those scenes, Enelesi and I spoke about her illness and Annie's care. She said to me as she glanced over at Annie: "When your heart is heavy with something, there is always someone who will control you. When there are children, they will tell you that it's all right and the sun will rise tomorrow." Children, Enelesi explained, helped manage the negative sentiments held by others, which might harm her or inhibit her recovery from TB and her management of HIV. Enelesi's statement highlights a particular aspect of the TB and HIV epidemics that I have not yet discussed, which is that Zambians sometimes attributed their afflictions in part to invisible or supernatural forces, such as witchcraft or Satanism, caused by a lover, neighbor, friend, or family member.²² My point is that children's emotion work can have a range of interpretations and effects on the ill, which deserve acknowledgment.

WHEN TB IS NAMED

I return now to Elesia, the mother of two girls—Abby and Chiko—who told her daughters immediately and directly about her TB diagnosis. They represent the only case in my study where a caregiver explicitly named TB. Their situation offers an alternative view that may serve as a challenge to Euro-American assumptions that naming disease has beneficial effects on both children and the sick. As I mentioned in the introduction to this book, Elesia's sister, brother, uncle, and other relatives gathered together just after Elesia's TB diagnosis to discuss who would take care of Elesia and the children. Elesia had been sick for a long time, but it was her TB diagnosis that prompted the family to separate Elesia from her children. The plan was contested by Elesia and her children, but eventually put into place when the family moved Elesia to her sister's house in another low-income settlement on the opposite side of the city. The girls moved in with their uncle (see chapter 2).

Several weeks after the separation, Olivious and I visited Abby and Chiko at their uncle's house. Tired of drawing, the girls asked for my tape recorder and started shyly singing into it. Once they tired of singing, Olivious suggested that they might take the recorder in the bedroom and *bisa* [hide] from us. That way, she explained, they could play more freely with the recorder. When they returned from the bedroom, they asked us to play back the following recorded conversation:

ABBY: Us here, we want our mother to come back and start living with her because we are used to staying with our mother. Chiko, come and say something.

CHIKO: My mother should get better.

ABBY: My mother should get better, I am praying for her so that we start living together, it's just her who is only getting sick.

CHIKO: We should also get sick.

ABBY: Why can't she just get better, she is just getting sick alone and being ordered around by other people. Me, I want my mother to get better so that we can start living together. I miss my mother a lot. I just want her to get better and come home. We are fed up of always going to visit her there. We want her to come home. We are used to staying with her and not other people.

CHIKO: What I dreamt today, I dreamt that people from the clinic had come to tell me that my mother has died.

Olivious and I were the people from the clinic. My heart sank after hearing that line, and I phoned Elesia immediately so that she and the girls could talk. The girls told her about Chiko's dream. She reassured them that she was taking her medicine and already feeling much better.

Abby and Chiko's situation was interesting because, at first glance and from a public health angle, they seemed to have several advantages over the other children. Their mother told them immediately that she had TB. The family put a plan in place for their care, and their maternal aunt took care of Elesia during the initial stages of her TB treatment when Elesia's care was very time consuming and her TB was possibly still infectious. Still, the situation looked quite different from Abby and Chiko's points of view. Even though they knew Elesia's clinical diagnosis, their inability to observe their mother's illness trajectory increased their concerns about their mother's recovery and their own future. The children expressed that they derived important benefits from being in close proximity to their mothers, namely the ability to ensure that she received care. In Abby's view, proximity rather than disclosure was the foremost indicator of good care for both her and her mother.

CONCLUSION

In activist, public health, and media reports, HIV disclosure has been viewed as a child's right, something that promotes an individual child's resilience and strengthens communication between adults and children and moves children to action. The problem in such discussions is not the virus *per se* but how illness and death affect children materially and emotionally. The solution is to focus on a particular type of communication that is assumed to prompt even further communication, which will then allay the negative effects of AIDS-related diseases and deaths on children as well as adults. This solution misses the mark in part because of its "immodest claims of causality" (Farmer 1999): that naming is helpful and not naming can cause more harm. Naming HIV (or not) figures as both the problem and the answer to adult-child communication, relationship building, and well-being. Such a view elides the pressing economic and political factors that shape how all people—young and old—communicate about suffering. As this chapter has shown, focusing only on naming attends to just one small aspect of communication strategies and ignores other forms of knowing and communicating.

The issue of disclosure is one that will remain at the center of debates on children's protection, provision, and participation in the context of the HIV epidemic. I suspect it will also become a prominent issue in TB programming because of the recent global attention to tuberculosis in children. To an extent, it has also been identified as an important topic in the study of cancer and other diseases in the United States and around the world. For this reason, I would like to conclude this chapter with three points that my study raises for future research on disclosure.

First, by calling attention to children's detailed knowledge and tactical use of disease names such as TB or HIV, I do not intend to suggest that the children were completely certain about the biomedical diagnoses of their relatives. I do believe that uncertainty and confusion around diagnosis and prognosis were part of the children's experiences, as the children vividly showed in their role plays. But uncertainty and confusion were also part of the experience for the TB sufferers themselves, other adult family members, health workers in George, and even me. People expressed uncertainties when a person became sicker when taking TB medicine and/or ARVs. They expressed confusion when HIV tests revealed contradictory results through time, or when TB could be found only in an X-ray and not in a sputum smear. And, while most people in George struggled with poor health during treatment, there were a few who recovered their health in miraculous ways. To borrow Abby's words: "They come back even better than they were before." This too raised questions about the validity of diagnosis and prognosis. Perhaps the research and discussion on disclosure has focused too much on certainties in a way that erases the lived experience of uncertainty within illness, particularly in resource-poor areas. For in most studies and reports on disclosure, disclosure is framed as a yes or no variable. As a yes

or no variable, we cannot understand how uncertainties shape the ways people know and talk about disease.

Second, and not unrelated, despite the confusion and uncertainty around TB and HIV, children living with ill adults often had nuanced understandings of TB and HIV/AIDS, as well as biomedical treatments and caregiving for both. Children's detailed knowledge of disease—expressed in the role plays and in their private tape-recordings—demonstrates the limitations of a view that considers children as unaware or focuses on naming as the only way of knowing. Disclosure is a process, rather than a discrete event on which all other knowledge and action is built. By focusing solely on naming disease, public health actors have assumed that disease knowledge is encapsulated in a name or precipitates from an event. Instead, disclosure comes in many forms and knowledge of illness processes is built through interactions.

My work calls for an extreme broadening of definitions of disclosure to include the silent presences and relationship-building practices, including the material objects used to communicate about a disease and specific forms of context-specific emotion work. As I have shown, children actively collected knowledge of diagnosis and prognosis. They gathered knowledge through attention to small signs, inscribed on their sick relatives' bodies and materially present in their homes. This suggests the importance of attending to multiple ways of communicating about disease, rather than just one. Drawings such as Annie's drawing of the four scenes of taking care of her mother raise a critical point. The drawings show the fallacy in assumptions that children are passive, uninvolved, and unable to communicate until they have been directly told about disease.

Finally, children make decisions on a daily basis about how and when to communicate about disease with family members, friends, neighbors, or a visiting anthropologist. They know the socially appropriate spaces to express their knowledge of TB or HIV and AIDS and where and when to remain quiet or use euphemisms and indirect references. This suggests that it is just as important to understand children's concealments as it is to understand what they do and do not know, or what they have and have not been told. Children experience and respond to a range of actions that stigmatize, blame, or shame people who are diagnosed with TB or HIV. As I have shown in both the guardian's and children's responses to naming TB, sometimes not naming, not knowing, or not confirming were the only ways in which people were able to hold each other close.

CHAPTER 4



Following the Medicine

George Health Centre is the only government-run health facility in George.¹ The clinic is set in the oldest section of the residential area, along a dusty, narrow, and deeply grooved road. It has one single-car entrance for a slow but steady stream of hired vehicles taxiing individuals too sick to walk, cars driven by the few nurses who can afford them, run-down Ministry of Health land cruisers, and expensive all-terrain vehicles belonging to internationally supported projects.² Most residents of George walk or are carried through this entrance. The very sick are pushed in wheelbarrows, with blankets protecting them from the morning chill and the gaze of onlookers.

Current policies on TB treatment make traversing between home and clinic a daily, weekly, and then monthly activity. TB medications are the lifeblood of the clinic's TB treatment efforts. In accordance with the World Health Organization's guidelines on TB control—directly observed treatment, short-course (DOTS)—TB medications do not flow freely from the clinic's pharmacy. Instead, a nurse doles them out on a regular basis and monitors their flow to ensure that patients do not discard, forget, or sell them over the long course of treatment. TB medicines come with many rules for patients to follow:

- Take in the morning.
- Drink with water.
- Eat plenty of good foods.
- Do not drink alcohol or smoke.
- Rest.

They come with similar rules for family members:

- Help the patient take their medicine.
- Be patient with them.

Encourage them.
Buy them healthy food and cook for them.
Let the patient rest.
Keep the patient and their surroundings clean.
Let fresh air circulate.
Above all else, make sure that the patient takes their medicine.

Even though TB medication has been around for a long time in Zambia and elsewhere in the world, TB medicines and the many rules that accompany them are, in many respects, new in George. Their presence is a result of substantial global and national shifts in TB control during the early twenty-first century. Treatment entered George with gusto as the nation aligned its TB control effort with global mandates at this time, and substantial international funding followed. Clinicians and community members in George opened their arms to these medicines and the rules that accompanied them, after struggling to manage the effects of TB for many years without sufficient supplies of medicine or other resources. In many respects, the rules of TB treatment brought a sense of empowerment. There was finally something clinicians, family members, and TB sufferers could do about an epidemic that was killing their loved ones.

I am interested in the social implications of such substantial attention to patient adherence to medication—not just for TB, but also many diseases in which medicines serve as the primary solution to prevention, treatment, or cure. More specifically, I join other anthropologists in examining how universal treatment programs have shaped everyday efforts to direct life projects (Biehl 2005; Koch 2006, 2013). Erin Koch (2006), for example, has shown the unexpected consequence of DOTS-based programming in a prison in postsocialist Georgia. She observed that prisoners were trafficking TB-infected sputum, the bodily substance that contains TB bacilli if a person has pulmonary TB. Prisoners who did not have TB obtained and used the infected sputum of TB sufferers to achieve positive TB diagnoses on regular sputum tests given by prison authorities. Such a diagnosis (when a prisoner was not caught) allowed the prisoner access to better housing, food, and medical care while in prison. Koch characterized such subterfuge as a form of agency. Universal treatment programs, as Koch has shown, can provide individuals in dire circumstances with resources to manage harsh realities.

Patients are not the only ones who use medicine to manage harsh realities. So, too, do families. In João Biehl's research in Brazil, he has suggested that, in a setting where medications serve as healthcare and infrastructure is defunded and poor, the family serves as the "medical agent of the state, providing and at times triaging care, and medication has become a key instrument for such deliberate action" (2005, 22). Family members make judgments and decisions about sick

persons based on a person's adherence to medications, creating and forestalling people's life chances.

I take Biehl's and Koch's observations as a starting point for a further suggestion: the materials and rules of TB treatment (in short, DOTS) offered children a form of agency in their families and a semblance of control within their highly marginalized positions. While researchers of TB, HIV, and other diseases have noted that children can be medicine promoters who encourage adults to take their medicine, my observations propose something much more substantial.³ Children were taking the rules and materials of treatment and making them their own, using them to shape their relationships to sick adults, their ties to households, and the resources they hoped to receive.

An understanding of children's engagement with global treatment technologies requires some background on the historical changes in TB control measures and how such measures have entered into Zambia and, specifically, George. I start this chapter with a brief history of the increasing pharmaceuticalization of TB.⁴ I then describe how DOTS-based practices have shaped TB medication distribution at George Health Centre and are affecting what it means to give good care to TB patients. After providing this necessary background, I will return to the children and show how they were taking the promises of and guidelines on treatment adherence and carrying them out in their homes. I will demonstrate the lengths to which they went and the meanings they associated with adherence. Such an analysis moves us beyond debates about the feasibility of involving children or other family members as observers of TB treatment adherence to an examination of how universal policies and their ideologies transform and become meaningful in local contexts and to particular actors.⁵ A focus on children's responses to TB treatment offers unique insight into how global public health technologies can shape and become a resource in children's life projects. In this case, the children followed the rules of treatment to safeguard identities, relationships, livelihoods, and futures that have become increasingly under threat in an era of TB and HIV.

A BRIEF HISTORY OF TB CONTROL IN ZAMBIA

At Zambian independence in 1964, the new government set out to create a "health service for the people second to none anywhere."⁶ This was part of a much larger expectation that Zambia was fast on its way to becoming a modern country with a place in the developed world (Ferguson 1999). TB—a disease that had declined significantly in developed countries by that time—had no place in their vision for their new country. It seemed to officials that, during colonial rule, the Zambian people had not adequately benefited from the pharmaceutical innovations in TB control that had occurred in the first part of the twentieth century.

CONTEXT FOR READING

Pages 104–106 are omitted from this reading. Here's what you need to know...

Hunleth recounts Zambia's TB history. After independence in 1964, the country built an ambitious TB program: mass BCG vaccination, mobile X-ray screening, isolation wards. Economic collapse and the HIV epidemic gutted it; by the 1990s Zambia ran out of TB drugs, and an unquantified number of sufferers died. In 1993 the World Health Organization declared TB a “global emergency” and standardized treatment worldwide as DOTS (directly observed treatment, short-course), a five-part strategy in which a treatment supporter, assumed to be an adult, watches the patient swallow each dose. Critics charged that this surveillance ignored the structural conditions of poverty, and programs shifted toward “patient-centered” and “community” approaches that drew family members into treatment support. In 2002, DOTS arrived at George Health Centre's TB Corner, where you'll pick up...

as family members are already playing substantial, informal roles in treatment support.

In George, the mechanisms used to distribute TB medications and monitor adherence represent a bricolage of global, national, and local approaches to managing treatment adherence. Zambia renewed its commitment to DOTS in 2002 after the turbulent period in the 1990s when the NTLP nearly collapsed due to healthcare system restructuring and the withdrawal of external donor funding for specialized TB programming.¹⁴ After the NTLP regained external funding and support, TB focal persons were appointed to government health centers to coordinate TB services according to the DOTS protocol. In George and other clinics serving large populations of TB patients, this restructuring translated into the establishment of TB Corners, physical and temporal locations for distributing TB treatment and monitoring medication adherence.¹⁵ Even though there were a number of private clinics of varying quality in George, the TB Corner at George Health Centre was the only place in George where residents could access free TB treatment in the settlement. Almost every diagnosed TB patient in George ended up in the TB Corner during treatment.

TB TREATMENT ON THE GROUND

The way people move through George Health Centre has everything to do with what ails them. Despite the singularity implied by its name, the clinic comprises eight buildings. Most people find themselves, at least at first, in the oldest and largest building. This building houses examining rooms, a maternity ward, a diagnostic lab, and a pharmacy, among other services. The other seven buildings have been constructed one after another over the past decades with international donor funds and for highly specific purposes: breastfeeding trials, syphilis and gonorrhea research, HIV and AIDS treatment, physical therapy, and, in 2008, TB treatment. In a nod to the verticality of public health programs and the popularity of disease-specific research in urban Zambia (and around the globe), many buildings retain the names of the former projects that funded their construction. Staff working on the Zambia and South Africa TB and AIDS Reduction study, with which I affiliated my research, enjoyed telling people that we could be found in “Breastfeeding,” a building constructed for a long-running and completed breastfeeding project carried out by a prominent research university in the United States.

When entering the clinic gate, Breastfeeding is the first building you see. It is located just in front of an older building that contains some offices, a staff cafeteria, and bigger rooms used for different reasons throughout the year. In the rainy season, its biggest room has served as a cholera ward. The driveway branches off between this old, multipurpose building and the main clinic building. At this

junction, and during the time of my study, a large sign announced: “Get your *vikolala* [sputum] tested here.” A ZAMSTAR worker sat beneath the sign, talking to people about TB testing. Moving straight past the sign, the ARV (antiretroviral) clinic is tucked at the back of the clinic grounds. A left turn at the sign leads to a covered ambulance carport that served as the TB Corner at George Health Centre from 2002 until 2008.¹⁶ The ambulance that once sat in the carport is long gone.

The TB Corner was not simply a space for treatment, but also a practice of treatment. TB drug distribution followed a similar pattern every day. As TB patients and their families filtered into the driveway and carport, the TB Corner nurse and TB treatment supporters, local volunteers trained by NGOs and the District Health Management Team, set up benches and tables. They brought out large containers of TB pills, a hardcover TB Patient Log that listed individual names, addresses, and HIV test results, and a number of paper charts. TB treatment supporters and nurses hurriedly divided pills from large bottles into bags of seven and twenty-eight pills.¹⁷ The patients who came daily for their pills were given a single dose to take at a water station.

Because patients were required to collect their medicine regularly, the TB Corner was packed on every morning of the week, with patients and their family members crowded into the carport and standing along the driveway and grass adjacent the carport. For the first two months, patients were required to collect drugs either daily or weekly. Sometimes, if a person was too debilitated or otherwise unable to come, other household members, kin, or neighbors came instead (a practice that was tolerated to varying degrees by TB nurses). At the end of two months of “intensive” treatment, the “continuation phase” of treatment began, which allowed individuals to collect their medicine monthly.

NO CHILDREN IN THE TB CORNER

Children were conspicuously missing from the daily bustle of TB Corner activities and TB treatment protocols during my fieldwork in 2005 through 2008, something that has been changing since the World Health Organization placed a spotlight on childhood TB in 2013. Children have not been diagnosed with TB as frequently as adults, for reasons related to the distribution of the disease and the difficulties of diagnosing TB in children, making most patients in the TB Corner older than fifteen. Their accompanying family members appeared just as old and older. Children’s absence from the TB Corner seemed to contradict their important and acknowledged role in running errands and responding to the needs of adults, as I discussed in chapter 2. For example, my survey of children’s roles across 200 households in George showed that running to stores and tuck shops for medicine such as Panadol (a pain reliever) was a primary errand that many

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children carried out during the month prior to my survey. Waiting in the TB Corner, however, was an adult errand.

I asked two TB treatment supporters why children in George were rarely sent to the TB Corner to pick up drugs for parents, grandparents, aunts, uncles, or neighbors. Their responses suggested that local concerns over children's risks might have something to do with this rarity:

MAXWELL: We look at the age. Maybe if they're fourteen or fifteen it's okay.

But not ten because we restrict. We don't allow children.

JEAN: So if one comes, what do you do?

MAXWELL: We say, "Go and get someone older than you. Here, there is TB here, it can be contracted from here." That's why we don't want children to collect. When we see them, we return them to go and call the "olders" to collect the medicine.

JEAN: Do they ever come back and say that the olders can't come?

GODFREY: No, if they do [come the first time], we do send them [to call someone older]. They do send someone older.

Given the worry over children, if a child was needed to pick up TB medicine from the TB Corner, she was most likely already exposed to the disease in the home. However, the TB Corner demonstrated publicly the possibility of TB infection for everyone. Family members, TB treatment supporters, and nurses sat in poorly ventilated sections of the clinic for long periods of time. Placing a child in such danger, in public, was viewed as unconscionable. TB was an adult's disease and treatment was carried out for and by adults. A view from the TB Corner may suggest that children were not exposed to global ideologies of TB treatment. A household view provides a very different story of children's participation in one of the largest global health efforts in the world.

MAKING CHILDREN'S ROLES VISIBLE

On a hot October afternoon, twelve-year-old Irene rushed toward me as I approached her house, asking: "Did you see my dad at the clinic?" Her usual excitement over my visits was replaced with a tone that conveyed anxiety and fear. Victor and Margrete, Irene's parents, collected Victor's TB medication at the clinic early every Tuesday morning. I had seen them waiting at the TB Corner that morning. However, it was now late in the afternoon and they should have returned. Irene and I stood outside of their mud-brick house with her nine-year-old brother, Floyd, looking in the direction of the health center, a two-kilometer walk on dusty paths that wended their way through tightly packed housing.

While Irene's parents went to the clinic weekly to stand among the crowds of other adults receiving treatment, Irene and Floyd waited at home. They carried

out household chores, watched their younger siblings, and generally worried that the clinic visits might indicate their father's worsening condition. That day, Irene and Floyd were selling bags of crushed stones, which Irene had helped her mother crush from a rock outcropping behind their house. Because crushing stones is such hard labor for its payout, this seemed a particularly desperate attempt to raise money to sustain the family. The family members made other last-resort decisions, such as agreeing for their older daughter to marry a man who had impregnated her, a decision that Victor and Margrete had avoided well after the birth of the baby because they were unsure that the union would be in their daughter's best interest. It turned out that it was not. However, the family's financial situation was dire. Both Victor and Margrete had to stop their work, Victor because he was sick and Margrete because she needed to care for him.

As the sun began to set, Margrete returned alone with Victor's medicine. She was angry. Victor had left the clinic hours before after becoming annoyed with the long wait to get both his TB medications and his ARVs. Margrete remained to pick up his medications, without which he would most certainly become sicker. Margrete's disposition changed from anger to concern when she learned that Victor had not returned home. Irene ran out of the house to look for her father and returned thirty minutes later with Victor limping behind her on a clumsy pair of homemade crutches. It had taken him hours to walk the two kilometers on his own. I left as Floyd helped Victor get into bed and rushed to get him drinking water. Irene went outside to cook dinner, and their mother slumped over in exhaustion.

The absence of children at the TB Corner gives a wrong impression of their involvement in TB management and in what the World Health Organization might consider as directly observed treatment. Irene and Floyd's presence at home during Margrete and Victor's weekly trips to the clinic facilitated Victor's TB and HIV treatment and Margrete's ability to give care, while appearing to be separate from it. A simultaneous view of household and clinic provides a more accurate picture of the real work that goes into collecting medicines on such a frequent basis. And it suggests something further, something that the children made evident in their role plays: the clinic and the household are not separate zones of activity. In the children's role plays on "living in a house with a sick person," they emphasized this connection, despite their restriction from clinical space of treatment. Across the nine role plays performed during three separate workshops with different children, all children acted out protracted episodes of walking to and from the clinic to collect medicine. Children demonstrated how the home, the clinic, and the paths in between were implicated in the care and treatment of the sick.

The children's accounts show that "being a TB patient" is a familial—rather than individual—condition. While treatment can be empowering, it is also

disciplining. Family members of the sick, as well as the sick, experienced this discipline in their regular trips to get medication from the TB Corner. Children experienced it in their efforts to make these clinic visits possible, through their work in minding smaller children, carrying out household chores or income-generating activities from home, and attending to the sick person, when he or she was left behind, too debilitated to make the clinic trip. As we will see in the following section, the collection of TB medications was not the only aspect of DOTS with which children became involved.

ENCOURAGING ADHERENCE

Global concerns about treatment “adherence” and “default” guide most aspects of TB treatment in George, as they do around the world. Unlike other countries, MDR-TB in Zambia remains low at less than 2 percent (ZAMBART 2011). For the NTLF, this figure situates drug resistance as both a future problem and a very real fear, given the high levels of drug resistance in close-by countries, such as South Africa. The concern over drug resistance is written into DOTS protocol and has led public health actors to recommend patient adherence to drug susceptible regimens as the most important route to averting resistance (Mulenga et al. 2010). The first part of ensuring adherence has been proving and proclaiming its efficacy. The second part is getting patients to take their medicine daily, even when they feel well. The routines around TB treatment in the TB Corner and the innovations of NGOs have served these purposes, while promoting a particular ideology of TB treatment adherence as a community and family responsibility.

Because of the years without treatment, people in George knew firsthand the efficacy of TB medicine and frequently recounted its efficacy, even when it had not benefited their family. I routinely heard that “medicines from the clinic” were what sustained life. TB sufferers and their family members listed home and over-the-counter remedies they used prior to TB diagnoses, but suggested that they stopped these once diagnosed. Prayer complemented biomedical TB treatment, but was not seen as a replacement for it. Only one person in my study admitted to me that she saw a traditional healer during her treatment. Her healer was treating the underlying reasons for all of the diseases from which she suffered, including but not limited to TB. This same woman attended the TB Corner daily.

Even though residents largely believed in the efficacy of TB medicine, not everyone took their medications according to clinical protocol. Taking TB medication was not always easy or possible. Direct observation of treatment had never really caught on in George because debility, distance, and work made it difficult for most patients to come to the clinic daily. Alternatively, resource shortages and the sheer number of TB patients made it impossible for clinicians to bring medicines to patients’ houses daily. Clinicians, NGO workers, and local

volunteers relied on a range of other materials and activities to promote adherence to medicines. At TB diagnosis, patients were handed their “green card,” a folded piece of thick green paper that classified them as a TB patient. Green cards had tiny numbered boxes that patients were asked to tick each time they took their medication for a total of eight months. The nurse reviewed their cards weekly or monthly to ensure that they had not missed a dose. While the cards were the most blatant attempt at monitoring and encouraging adherence, there were many more efforts. Each morning in the TB Corner, activities commenced with a Christian prayer and an educational session, during which the TB Corner nurse, community volunteers, and NGO workers spoke on issues of TB treatment adherence and how to be a good patient and, for family members, how to take good care of a patient. These sessions included explanations of how TB medications worked, with emphasis on the importance of adherence and advice for living a healthy lifestyle.

NGOs working in George experimented with a range of other strategies to enhance adherence to treatment. Alongside the global emphasis on patient-centered TB treatment, their efforts tended to focus on the care needs of patients, and particularly the role of community and family in promoting care. A Japanese NGO known as AMDA-MINDS, for example, trained a set of local community volunteers in George to serve as TB treatment supporters. They helped in the TB Corner, visited patients in their homes, and spread TB treatment messages around the community. The NGO’s website explained the TB treatment supporter program in a brief entitled “Zambia: Anti-Tuberculosis Program through Community DOTS in Lusaka Compounds”:

Since TB treatment requires rigid drug adherence and failing to do so leads to a huge risk of producing multidrug-resistant TB, the patients need support from family members and neighbors while coping with social discrimination and stigma. This program is establishing a model to protect communities by the communities. . . . As awareness and understanding on TB among [George] compound residents have been enhanced, and drug adherence has improved, the TB mortality rate has reduced.

In such a perspective, awareness, education, and an encouraging family and community environment, rather than direct observation, were the tools for cultivating adherence.

The twin ideas that TB medications were efficacious and patients needed support and encouragement were encapsulated in graduation ceremonies conducted for people who completed their treatment. AMDA-MINDS supported the graduations as a way to celebrate patients’ recoveries from TB, viewing recovery as an achievement facilitated by the clinic, TB drugs, the assistance of TB treatment supporters and family, and an individual’s hard work. The graduations

were joyous events for the people who attended. However, not one of the eighteen TB sufferers in my study participated in a graduation ceremony. Perhaps the significance of the graduation ceremonies was lost on them because most did not have a TB treatment supporter who regularly visited them. The numbers of TB patients that TB treatment supporters were supposed to follow was large and their efforts voluntary. As a result, TB treatment supporters rarely saw all of the patients they were assigned, and the largest sources of support continued to be family.

While attempts to form a sense of community and accomplishment around TB through graduation ceremonies might have fallen short, the graduations served a less obvious but more significant purpose. Because the building in which they took place was adjacent to the TB Corner, they were conducted in plain view of current TB sufferers receiving their daily, weekly, or monthly allotments of medication. Clinic workers told me that graduation ceremonies provided these people, who were still on treatment at the TB Corner, with the encouragement they needed to keep taking their drugs by acknowledging that others had survived. Encouragement became a strategy used in the TB Corner and beyond to foster both adherence and hope for the future. This focus on encouraging adherence solidified the role of family and community in treatment, whether they were official treatment supporters or not. Children, too, understood and conveyed messages on treatment adherence that came out of the clinic and circulated in their households and community.

“IF YOU DON’T FOLLOW THE MEDICINE, YOU CAN DIE”

Across my various methods, children emphasized the importance of “following the medication.” They used this phrase to refer to taking medicines every morning until treatment completion and also to convey the complex of activities required of patients when on medication. These included attending the clinic, eating a nutritious diet, resting, avoiding strenuous chores or work obligations, and eschewing alcohol and cigarettes. When I asked them about TB in my children’s workshops,¹⁸ the children invoked these rules:

MUSA: If you finish your medication and you start drinking beer you can suffer from TB again.

RHODA: If you are taking TB medication and start feeling better, then if you stop, you won’t get better or cured.

Going to a spiritual healer or *nganga* (translated as traditional healer) for TB was not part of the children’s understandings of the rules of treatment. When I asked the children if people at times sought care from these other providers, they answered with a resounding “*awe* [no].” In one group, twelve-year-old Irene

shifted in her seat after her loud “awe,” admitting quietly: “Some people when they have TB they don’t come to the clinic, they prefer going to the nganga and [they also prefer] saying that they’ve been witched. But it’s really TB.” Repeating widespread public health messages, children suggested that traditional healers who made promises to cure TB or HIV “just want money,” “they don’t tell the truth,” and “they can’t heal you.” “When you go to a traditional healer,” Musa said, “he’ll ask for one million *kwacha* and tell you lies.” Traditional healers in George acknowledged that biomedicine was the preferred TB treatment option, even for many of them. As one traditional healer phrased it: “When they know that these are TB symptoms, people usually run to the clinic. Unless in those areas where the clinic is far or there is no clinic, then the person will go to the traditional healer first.”¹⁹

In a different conversation, Musa extended this view to the multitude of private clinics that have cropped up in George, and which took his own brother’s money while he continued to get sicker. There were extreme consequences for not following the precise rules of treatment or for seeking care outside of the DOTS-organized clinical settings. The children broached these consequences directly in general statements such as: “Not following TB medication, you can die” (Sarafina); “If you don’t take the medicine given to you from the clinic, you can die” (Irene).

Chiko took adherence to the rules of TB treatment one step further than other children in my study and, in doing so, showed how family members are also subject to the rules of TB treatment and can be considered adherent or not. One standardized rule of TB treatment is that all pulmonary TB patients in Zambia and elsewhere are required to submit follow-up sputum samples two months into their treatment. This follow-up test, in which patients give sputum three mornings in a row, alerts health practitioners about the efficacy of the medication. If bacilli are not found, it marks a transition from intensive to continuation phase medications and enables a person to collect their medications monthly, rather than weekly. Instead of submitting sputum for her follow-up test, Chiko’s mother, Elesia, continued to collect intensive phase medication weekly from the clinic. Elesia told me that it was just too painful to walk so early to the clinic and produce the sputum needed for the test. As Elesia spoke to me about her struggles to produce sputum, Chiko interrupted, telling Elesia that she would give the doctors her own sputum for the test. Elesia’s lack of adherence to the diagnostic rules of TB and her continued use of intensive phase medications distressed Chiko as well as her sister, Abby, who wished to usher their mother through treatment to recovery. The delay symbolized a prolonged illness and a looming threat of further separation or their mother’s death.

The children’s notion of following the medicine offers insight into the powerful juxtaposition of biomedicine, personal and familial responsibility, and

recovery. The emphasis that children placed on treatment adherence speaks to the tremendous success of public health campaigns in advancing messages about TB treatment and making medications available. TB treatment held power in children's imaginations because policies that promoted biomedical TB treatment have changed the prognosis of TB for many sufferers. Before viewing children's emphasis on following the rules of treatment simply as repetition of public health messages, we must consider the moral and social assessments and distress in their statements.

MEDICINE IS CARE

Because medicine is equated with cure and there are specific rules to follow to ensure this cure, the practice of following the medicine has become entangled with notions of good family care for TB patients. Recall eleven-year-old Rose's tape-recorded story from the previous chapter, in which she referred to a picture of a person in a hospital bed, surrounded by family members. In her story, she described a father who was sick with TB. Speaking in the voice of the father, Rose said: "I am sick my children. But my children, I am taking my medicine, I went to the hospital and I was given this medicine. In case I might forget my dosage, you should remind me. And they have also given the vitamins. These are the TB tablets. I was also told to eat a lot of food and fruits." In this passage, Rose's father showed himself to be an adherent patient, willing to take his medication and make other adjustments to get well. However, he also suggested that his children had a role to play in supporting his adherence.

Rose's story demonstrates, vividly, how people domesticated the rules of TB treatment in their efforts to show care to loved ones. The process she described was twofold. First, family members showed their care for a patient through helping and encouraging them to adhere to their medicine. And, second, patients reciprocated this care—and showed care themselves—when they made efforts to follow the medicine. At the end of Rose's story, Rose says in her own voice: "He later got better because he followed his medication very well. My father didn't die." This last statement is important because it shows that his death would be indicative of a lapse in care for and by her father, and not a failure of the medication.

Patients invoked the support they received in taking their medication to both express and assess the quality of care they received from family and community members. This became evident in a survey I conducted with TB sufferers during the initial months of their treatment. I was interested in understanding how TB sufferers experienced social support or abandonment, so I asked them to list people who knew about their TB diagnoses and how each person responded to the news of their diagnosis. Munyongo's response to my survey was representative of

the majority of responses I received. Munyongo lived alone with her only child, Sarafina. Her husband had died of TB years prior to Munyongo falling ill. While the pair lived alone, they had family nearby in George and also farther away in other residential areas in Lusaka and the rural areas outside of the city. Because she believed that Sarafina was unaware of her TB diagnosis, she did not list Sarafina in her survey response. Here is her list of people who she said knew about her TB and her interpretation of their responses to her diagnosis:²⁰

1. [Maternal] uncle: He encouraged me to take the medication and not to miss one dosage of the drug.
2. Elder brother: He also encouraged me to take the medication.
3. Young brother: He also encouraged me to take the medication.
4. Elder sister: She did not want to see me. Even now she does not come to see me.
5. Church members: They encourage me to take the TB drug and they were always visiting me.
6. Cousins: They encouraged me to take the medication and told me that I will be okay.

Munyongo identified encouragement as an act of acceptance. Her elder sister's refusal to come see her was emblematic of a breakdown in the relationship and a lapse in care.

In a similar account, another woman in my study who had TB, Sarah, emphasized her social abandonment in her list. She said that most of her family members “didn’t say anything” about her TB when they found out. Only her twelve-year-old niece, Tracy, who moved in to Sarah’s house to care for Sarah during her illness said something when she found out about her TB.²¹ In Sarah’s words: “She encouraged me to take the medicine.”

Only people who cared enough encouraged a person to take their medication. In the previous chapter, I suggested that the children’s role plays on “living with a sick person” demonstrated children’s knowledge of when and where TB was named. I described the role play enacted by Musa (doctor), Rhoda (aunt), Sarafina (mother), and Gift (sick man) as an example. I did not describe how that role play ended. At the eleventh minute, Musa, still playing the doctor, drove his car across the Demonstration Room to where the sick man was living in order to find out why the man was not recovering. The implicit message was that something must be wrong in the family that was preventing the man from recovery. Upon arriving at the house, the doctor learned that the man was living with his maternal aunt and not his mother. The doctor made the following assessment:

So you, the mother, you want your son to be drinking his medicine from his auntie’s place. His aunt won’t be reminding him on which time to take the medicine. The aunt won’t even care. She’ll just be saying, “After all, he’s not my

son. That's the reason why his mother chased him [away]. So me I don't care whether he takes his medicine or he doesn't." So just let him take his medicine from here. When he finishes, we'll see how he's going to be.

The tone of the doctor's last sentence was particularly ominous, implying that, even after the man finished taking his medication, he would remain sick because his aunt did not care enough to monitor his medication.

Debates about the proper nurture of the sick often revolved around both the positive and negative sentiments of caregivers, namely mothers, and also the children themselves as caregivers. The children witnessed gendered conflicts in their households and families when sick family members' did not recover quickly enough. These debates occurred during illness and after death. In the role play, Musa, who played the doctor, enacted the debates he witnessed in his own family when his brother moved from his aunt's house to his mother's house. Such debates did not end at death. Particular women, including Musa's mother, received the blame for not monitoring treatment carefully enough. Musa's words offer insight into more than the gender inequalities in care, which I will discuss in more depth in chapter 5. They represented the children's views of their own roles in the care of sick persons and what they perceived as important to ensuring their ill family members' recovery. The children cared enough to make sure that the sick person took their medicine. They accomplished this by remaining close to home.

Adults praised and encouraged children's home-based involvement with medication. When I first met Moffat, a man who suffered from TB and was also on ARVs, he sent his eleven-year-old son, Stephen, to the bedroom to show me his medications. Moffat smiled with pride as Stephen explained when Moffat was supposed to take each pill, showing that Stephen's attentiveness was also meaningful to him. Even if TB did not fit neatly into ideas about childhood, the medicines and rules were available to children. Getting medicine and drinking water not only aligned with DOT, but also fit well with children's socially ascribed roles in households, in which a good child is considered to be one who is responsive to adults. Conversely, a child who does not listen to adults' commands or who stays away too long is considered naughty. It was no surprise, then, that when I asked adults with TB about the changes in their children their most common response was that the children wanted to stay closer to them than before their illness. Staying close was an emotion and practice of care that the children demonstrated through their attentiveness to the sick person's needs. Both children and adults identified the quality of a child's care by invoking the child's proximity and attentiveness, and their willingness to bring TB pills and drinking water for taking the pills.

When the children's parents and guardians became sick, the children positioned themselves to be the family members who reminded them to take their

medication. The children showed themselves as particularly knowledgeable about TB treatment adherence and the best ways to encourage and convince someone to take their treatment. Their views are, perhaps, best explained by Abby, Alick, Mulenga, and Agnes in the role play they created on “living with a sick person.” All four children were living with an adult who was currently taking TB treatment, two mothers, a father, and a “sister” (maternal relative). In their skit, a young girl’s mother, played by Abby, started coughing violently. Agnes, who played the daughter, took a break from washing the dishes to ask Abby: “Mommy, what is the problem? Why are you coughing, coughing? Your cough isn’t finishing [getting better]. Do you have a 500 [US\$ 0.10] so we can go to the clinic?” Abby handed her daughter the money as Alick, Abby’s husband and Agnes’s father, returned from work. At the clinic, Abby received a diagnosis of TB and her daughter and husband were given pills. The clinician gave them instructions on how Abby should take them, and he alerted them to watch Abby take them. They returned home, with Agnes keeping careful watch over her mother’s medicine. Her concern over her mother’s proper use of the medicine culminated in her father attempting to take over her duty of providing the medicine. When Alick motioned as if he were giving medicine to Abby, Agnes yelled: “No, Daddy! It’s only one [tablet]!” She grabbed the medication from her father and reinstated her role as the primary treatment observer. The role play continued with Alick playing the clumsy husband and Agnes the knowledgeable young girl in charge of treatment. As opposed to the role play I described earlier, this role play centered on a sick mother. Because of this, the young girl played an especially significant role in caregiving, which included monitoring medicine, a topic I explore in depth in chapter 5.

Children’s self-ascribed roles as treatment supporters became most evident during the times when their efforts were constrained. Consider nine-year-old Luka and eleven-year-old Paul’s situation. In 2007, Luka and Paul’s father, Musonda, started to cough, lose weight, and suffer severe body pains. His legs swelled to twice their usual size, and he could no longer breathe easily. Accompanied by his wife, he made several extended visits to George Health Centre, where he was referred to another health center, farther away from his home, to have an X-ray of his lungs.²² The X-ray confirmed the suspicions held by Musonda and his family that he had pulmonary tuberculosis.

After Musonda’s diagnosis, he and his wife began collecting his TB treatment at the TB Corner. Luka and Paul stayed at home during these clinic visits, but they were in charge of bringing Musonda his medication with a glass of water each morning. It is not easy to take TB drugs when you are sick, and the TB pills are particularly large and difficult to swallow. As Paul once told me, it took particular persistence to get his father to take the medicine each morning when his suffering became too severe, and his father expressed that dying was a desirable alternative.

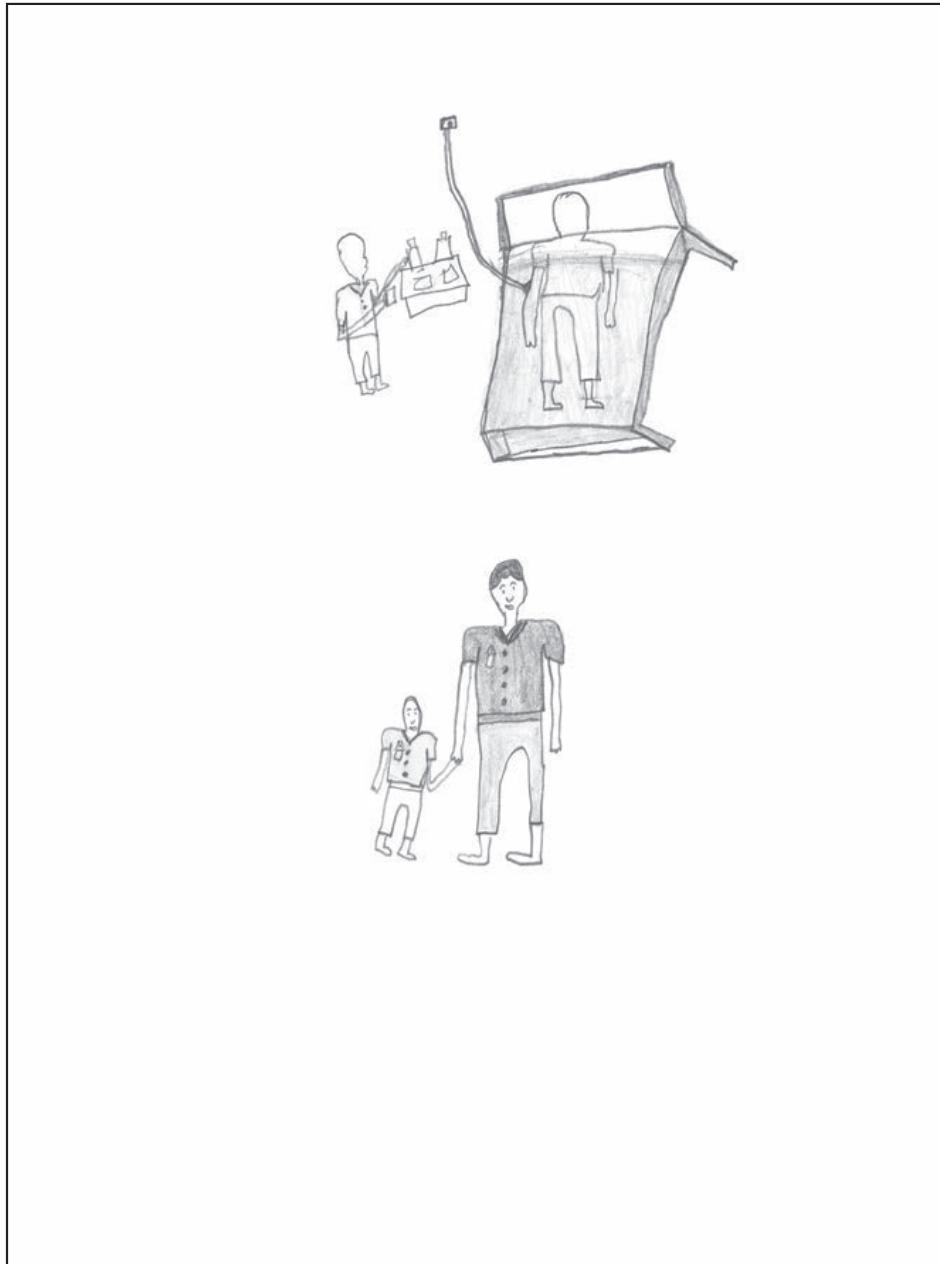


Fig. 4.1 Stephen taking care of his father in the hospital. Stephen accompanies his father on the journey back to George after he is discharged from the hospital. Drawing by Stephen, age eleven.

Six months after Musonda was diagnosed with TB, his condition became so dire that his wife (the boys' mother) took Musonda to the University Teaching Hospital (UTH), the main hospital in Lusaka. From George, UTH is about a one-hour bus trip with a transfer between buses in the city's center. When I heard that Musonda was in UTH, I went there to check on his condition, bring him some food, and see if his wife needed any help. Musonda and his wife were in the male TB ward, a large room with rows of beds. He had been diagnosed with a severe case of pneumonia, but he was given a bed in the ward because he was still on TB treatment. When I left the hospital, I drove to his home in George where I delivered news of his much-improved condition to the children who were waiting at home with relatives who lived nearby. Luka and his brother took my recorder outside and directed messages quietly into the machine. Luka said:

My dad is in hospital and I feel tired. And since the time he went to hospital, he has not come out. . . . So I asked the doctor if we [Luka and Paul] are allowed to go to [ward number] E21. He said "no." I don't know what to do. Maybe to go through the window? Children are not allowed and the building is tall. So doctor, take care of my father because we won't be coming to see him with medicine every day in the morning before he eats anything.

When I listened to Luka's recording that night, I was struck by his version of adherence, which suggests that children, rather than medical professionals, were responsible for ensuring adherence. In a reversal of clinical roles, Luka told the doctor to attend to his father's medication because "we won't be coming to see him with medicine every day in the morning before he eats anything." Just as the children's role plays showed, caregivers must care enough about the patient to remind them to take their medicine, and the relationship between treatment supporter and patient matters.

Luka's concerns related to his inability to enact care and ensure adherence. Stephen, Moffat's son, expressed similar concerns when Moffat was in the hospital and Stephen was also not allowed to visit. Instead of recording a message, Stephen gave me two drawings made on a single sheet of paper (see fig. 4.1). He described his first drawing: "Daddy is in hospital. He had a problem with the legs and he had a drip of water. I am giving Daddy medicine." In the next drawing, Stephen was holding his father's hand and, in Stephen's words, "Daddy is leaving hospital and he is going home with me holding his hand." I had visited Moffat in the hospital and the scene of the drip and medicine looked strikingly similar to Stephen's drawing. When I asked Stephen how he knew what to draw, he told me that he asked his mother to describe what the hospital looked like. It seems to me that Stephen continued his care for his father, only this time through his drawings. The drawings are a powerful reminder of Stephen's effort to remain present in his father's care when his ability to give care was foreclosed.

TB TREATMENT AND THE CATEGORY OF CHILD

I have shown in the preceding sections that, in George, DOTS-based programs and the insecurities faced during illness were converging to make observation of treatment (or DOT) a part of what it meant to be a good family member, neighbor, or community member. Children demonstrate that kin and age categories are reinforced, performed, and contested through daily TB treatment. By expressing concerns that relatives, doctors, and other family members might not care enough to encourage or remind their sick parents or guardians, the children were showing that they themselves cared enough. The result was not only hope that a sick person would recover, but also the reinforcement of their belonging in particular households and to particular people.

In chapter 2, I described Musa's experience of being sent to live with a relative on the Copperbelt and how Musa ran back to his mother's care. This was not the first time that Musa took such an approach to his living situation. As a reminder, I first met Musa just after his twenty-five-year-old brother Patrick had been diagnosed with TB and HIV. Following the diagnosis, Musa was sent to live with a relative, whom he called "uncle," a kilometer away. Musa's mother, Judy, reasoned that this move would relieve strain on household resources and protect Musa from TB infection and the burden involved in caring for the extremely ill. Patrick had been the breadwinner for the household, which included six people. Judy was able to make some money by selling hand-knit decorations around George, but she relied on Patrick's substantial inputs. Judy explained to me that they had used all of Patrick's money to seek treatment at private clinics in and around George before he was finally diagnosed with TB at George Health Centre. Plus their rental unit was small, with two rooms, a sitting room and a bedroom just big enough to hold a bed and some shelving. They reorganized their house and daily routines to accommodate Patrick's additional needs. Nights, Judy explained, were the hardest on the household because Patrick had frequent nightmares. When he had to use the outdoor toilet, the entire household woke to clear him a path through their makeshift sleeping areas on the floor and couches and to carry him outside.

To an outsider, Musa's daily life in his uncle's house might have seemed more desirable than remaining in his mother's household during Patrick's care. His uncle was better off than Judy, and he lived in a larger house with leisure items such as a television, radio, and video games. When I asked Musa to draw a day at his uncle's house, he gave me three pages of drawings, in which he presented a detailed account of a day consisting of riding a bike, playing video games, watching a baby, watering the garden, and sitting alone, "just thinking." He was alone most of his days there and had developed a close bond only with one young boy who lived in and carried out work for the household.

After a week in his uncle's household, Musa moved back to his mother's household. According to Judy, he simply showed up and started helping with house chores. I asked Musa why he had returned and if something had happened at his uncle's house to prompt the move. He replied that he needed to help Patrick take his medication.

Patrick did need people to help him take his medicine. He could no longer walk or even sit up on his own. During the last month of Patrick's life, my visits to the household revolved around Patrick's medical regimen. Musa discussed with me the times when they could or could not get Patrick to follow the medication. Patrick was no longer eating, something he needed to do to gain strength and because the clinic rules of treatment emphasized proper nutrition. Patrick's nightmares were worse, which Musa attributed to his medication and actively worked to interrupt by comforting Patrick. Musa slept in the household's only bed with Patrick, stayed near the room during the day, and noticed the smallest changes in Patrick's breathing and movement. Musa's persistence in caring for Patrick legitimated the time he spent with his brother and enabled him to remain in his mother's household, even though he had initially been pushed away.

Musa and other children helped with the medicine because they loved their family members, but their work as medicine supporters should not be considered selfless devotion to family. Instead, I argue that it served as one way in which children attempted to ensure the reciprocal provision of resources and love and preserve their identity as a loved child. Recall in chapter 2 Musa's reasoning for running away from his relatives' house on the Copperbelt. He viewed his mother as an important person who would advocate for his well-being when other relatives might not. His reasoning in that instance is informative for understanding how he interpreted being sent to his uncle's house. In his uncle's house, he risked becoming a child who was in the house but not of the household.²³ This, he noticed, had already happened to the young boy he befriended there, who carried out household chores, but without the reciprocal love and assistance that children typically expect of a good caregiver.

Musa worried that losing proximity to his mother and Patrick would make him an orphan and thereby alter his identity in damaging ways. This loss was more damaging than his potential loss of health. In a different household, as I showed in chapter 3, Chiko became so frustrated when relatives separated her and her older sister Abby from their mother after their mother's diagnosis that she suggested that she and Abby should also get sick with TB so they might be reunited. In this case, the loss of a beloved advocate was viewed as more deadly than the loss of health. The shadow side of the illness of an advocate was the potential social death of the child.

CONCLUSION

The introduction of universal TB treatment in Zambia has saved lives and demonstrated the efficacy of biomedicine. In its implementation in George, DOTS programming has combined the dual notion that TB treatment must be adhered to and that community and family bear responsibility for adherence. This social side of “following the medication” that has arisen under DOTS and within an overburdened health system is part of a larger pharmaceuticalization of public health (Biehl 2005; Biehl and Petryna 2011) occurring in Zambia where the state’s TB management programs now hinge on providing treatment and outsourcing care to families and communities. We need a richer understanding of how such a shift reconfigures social relations, reinforcing inequalities or becoming a tool to redress the harsh conditions produced when poverty and illness combine.

Several aspects converged to shape the ways in which children became involved in TB care: current modes of TB treatment delivery, the needs experienced by households when someone had TB, understandings of what it means to be a responsive child, and children’s efforts to create for themselves a good childhood. While TB treatment could be the fault line on which relationships were broken and family members cast out (Biehl 2005), it also represented a resource to negotiate belonging at a time when relationships and futures seemed most fragile. The ways in which the children utilized adherence models suggests that pharmaceutical resources had become indispensable to their efforts to make claims to households, people, identities, and belonging. The children’s actions were part of a broader landscape in which community is increasingly experienced and citizenship claims are made on the basis of biology and therapy (Nguyen 2005; Petryna 2002; Rose and Novas 2005). However, the children were experiencing belonging and enacting claims not on the basis of their own biology or therapeutic need, but through attending to the therapeutic needs of loved ones.

Biomedicine is localized in everyday life, its technologies and categories invoked and contested to make sense of experiences of extreme suffering (Ross 2010) and to recast relationships and responsibilities. As João Biehl and Adriana Petryna have observed, “It is at the intersection of the therapeutic imperative, the biotechnical embrace, and the reason of the market that the intensity of survival becomes visible” (2011, 381). The concept of biotechnical embrace, Mary Jo Del-Vecchio Good suggested, implies the affective response associated with biotechnologies. In her words, “‘embracing and being embraced’ fundamentally links contemporary high-technology medicine and bioscience to the wider society” (2001, 399). The children’s embrace of TB technologies became evident in their adherence to rules of treatment, their belief in its efficacy, and the hope it afforded for their futures. TB treatment, because it is so highly effective and requires stringent adherence, shaped their actions and imaginations. And yet,

the verb “embrace” is not strong enough. It does not capture the urgency and desperation in their attempts to sustain life, both social and biological.

Over the course of my research, it became clear that the children were neither marginal actors within the national and global programming based on DOTS, nor were they passive victims of the TB epidemic. As TB treatment has become a domestic project within the context of the state’s implementation of DOTS, children have also worked to domesticate TB treatment to gain control over, make sense of, and remedy adverse circumstances. Children strategically repurposed TB treatment regimens to reinforce kin relationships and social positions in households. They did this through actions such as reminding loved ones to take medications, showing themselves as knowledgeable about the rules of the medication, and situating themselves close enough to TB patients to administer medication. Carving out their own roles in TB care was an attempt to shape the care, love, and help available to them. This was not an easy task given the transmissibility of TB and the heightened concerns over and reality of children’s risks of infection.

Attention to children’s roles in TB treatment regimens exposes the forms of life that are forged at the intersection of public health, DOTS, illness, and childhood. As Ross has argued of belonging in a poor settlement in Cape Town: “a life worthy of the name is created by being dependent on others, and that dependence is precarious” (2010, 205). By considering children’s roles in TB treatment, we see not just children’s dependence, but how some children nurtured dependence through their attempts to create interdependence. The interdependence cultivated by children through TB treatment demonstrates one way in which children engage dominant discourses of global programming, such as DOTS, in resourceful ways that contribute to their sense of belonging to households and kin groups. Such an analysis suggests that the rubric of “treatment support” inadequately captures the meanings of adherence rules for children. Instead, making adherence possible became imperative to cultivating family, community, and childhood under conditions where security in relationships and resources were neither assured nor expected.

CHAPTER 6



Children and Global Health

Several months after Abby's mother was diagnosed with TB, Abby spoke the following words into my recorder: "When someone is sick in a family it affects everyone. They get worried. Some start thinking that the person might never recover or might die. Some worry because they don't have money to buy a coffin if she or he dies." Abby's close tying of sickness to death demonstrates the violence wrought by economic downturns and neoliberal projects, which have affected the state's capacity to mitigate such large-scale suffering and the capacities of families to care for the ill, in life and after death.

TB, other AIDS-related illnesses, malaria, malnutrition, substance abuse, and violence have severely reduced healthy life years in George. This reality was brought home to me every day when I walked through George Health Centre's entrance. And, in the past few years, a number of people who appeared throughout this book have died: Moffatt (Stephen's father), Munyongo (Sarafina's mother), Victor (Irene and Floyd's father), Musonda (Paul and Luka's father), Enelesi (Annie's mother).

Shared vulnerabilities accompany illness and death. Abby made these vulnerabilities evident as she continued her recording: "It would hurt me very badly if my mother had died when she was sick with TB." She quietly asked: "Who would take care of me? As to my side, I always pray to God to let my mother take care of me at least up to a stage where I can take care of myself and do the same for my sister and my brother. Not her dying, leaving me at this stage. No! No! Because some aunts say: 'They are orphans, I will do whatever I want to do with them.' That's what happens to orphans. They later become street kids." Abby connected her mother's possible death to her own social death, personalizing the representations of childhood created within the humanitarian response to HIV.

Yet Abby also punctuated her observations with optimism, saying: “Sometimes you might get very sick. Then people start saying that you are going to die, but surprisingly you recover, then gain weight even better than the way you were [before you became sick].” She concluded her recording with the words: “Life is wonderful. Everyone wants to live because life is wonderful.” Abby’s emphasis on recovery and life revealed emergent hope. She hinted at a newer aspect of the geography of infectious disease in Zambia and elsewhere—the growing presence of medical technologies that, at least in dominant ideology, sustain life and make a person even better than they were before they fell ill.

The second chances at life that TB and HIV treatment have offered some people in George are notable. Take for example Sarah, whose niece became her “mother,”¹ and Maureen, who asserted that her children provided most of her care, even though everyone expected it was the “elders.”² During my fieldwork in 2007 and 2008, I witnessed both Sarah and Maureen as they recovered their health in ways that were unexpected, almost miraculous. When I visited in 2014, Sarah appeared in even better health than when I last saw her, and Maureen was so busy with work that we were not able to arrange a visit.

It is little wonder that issues of both living and dying preoccupied Abby and other children and shaped their caregiving strategies. In George, medications are available as they never have been before. At the same time, health services are severely strained, and individuals and families must go to great lengths to receive care for TB, HIV, and many other conditions. Throughout this book I have shown that, within such a context, children refashioned biomedical materials and knowledge, as well as child protection discourses and local understandings of children’s roles, to fit their circumstances and needs. At the heart of understanding how they did so and what was at stake is the notion of care—both the care children gave and the care they attempted to receive.

A focus on children’s caregiving offers an alternative heuristic for understanding dominant themes in global health and humanitarian work, such as familial caregiving for children and the sick, intergenerational communication about illness, the pharmaceuticalization of public health, universal treatment programs, and gender inequities in illness and care work, among others. In what follows, I reiterate the major lessons I have learned from working with children in George, and I offer new insights. Because I believe strongly that children’s perspectives are needed for a fuller understanding of global health problems, I also outline approaches to include children in future global health studies and work.

REPRESENTING CHILDREN

One of the many challenges I faced in conducting research with children was how to consider children’s perspectives in the context of a stigmatized, “adult” disease and in realms where their views have been traditionally excluded from



Figs. 6.1 and 6.2 Two of many drawings of women's fashion that Irene made for me. Drawings by Irene, age twelve.

anthropological analyses, such as in caregiving and therapy management.³ It is not easy to learn from children in any setting, but it is particularly difficult within settings of adversity. The ethical issues I faced were many, as I have acknowledged throughout the book and most fully in my discussion of naming TB. As I was leaving Zambia to return to the United States in 2008, I was reminded that many ethical challenges remain in representing children's lives to a wider audience.⁴

In September of that year, I attended a Lusaka-based TB conference for practitioners who worked with the Consortium to Respond Effectively to the AIDS/TB Epidemic, a multi-country effort. Since 2005, I had worked sporadically outside of George on TB-reduction programs implemented by researchers from the Zambia AIDS Related Tuberculosis Project. We were presenting our findings

related to an intervention that included schoolchildren in TB reduction activities in a number of sites around Zambia.⁵ The presentation included drawings that my colleagues and I had asked the schoolchildren to make during the final assessment stage of the project. In those drawings, the children depicted TB and HIV services in their communities.

My colleague, Virginia Bond, presented the materials, flipping through slides of children's drawings of clinics, TB Corners, and HIV testing services. She paused on a particular drawing to explain its significance. It was a particularly artful drawing, and I noticed many people smile when it appeared on the screen. The man seated next to me leaned over and whispered: "Now that one is going to be an architect." I might not have taken note of his comment had I not received a range of similar comments after the presentation about how good the drawings were. Such comments centered on children's skills and potential future professions, glossing over both the severity and importance of what the children were actually depicting. These are comments that I have received in other forums and are not particular to the people who participated in this meeting. Even I found myself awed by the skill with which twelve-year-old Irene executed her drawings of women's fashions (figs. 6.1 and 6.2).

In reality, the children participating in the schoolchild intervention, just as the children in my study, were unlikely to become architects and fashion designers because of where they were born. The ease with which we consider children's drawings in the future tense ignores this injustice. Aesthetics can be distracting, eliding a political and economic analysis of the worlds in which children live. The comments betrayed something equally as insidious. It is impossible to take children's knowledge and experiences seriously when they are viewed as becoming rather than being. If we only look at children's drawings with a view of the future, we cannot take seriously the challenges and hurts they face as children in the present, and we cannot take seriously what they anticipate for their own futures.

BEYOND CHILDREN-AS-VICTIMS: TOWARD AN UNDERSTANDING OF CHILDREN'S HEALTH AGENCY

There is no denying that children living in heavily TB- and HIV-affected areas are, in many senses of the word, victims of global processes that have sustained inequalities in health and resources. At the risk of sounding repetitive, I describe again the many factors that have negatively affected the children in my study and were clearly out of the children's control. Well before the children were born, a declining economy, neoliberal policies and structural adjustment programs, increased urban poverty, a gutted healthcare system, the emergence of HIV, among other factors, created conditions ripe for a sustained TB epidemic. International policies in the 1990s led to a shortage of TB treatment in Zambia as well as the inaccessibility of ARVs (for HIV) for most citizens until 2004, even as ART

had been available to many people living in wealthy countries for years. Policies and funding streams have changed, and there are glimmers of hope, but the roll out of ART and the greater availability of TB medications alone have not ended suffering, especially for the most marginalized.

Viewing children as victims of these processes, however, is overly simplistic. It casts children as passive and dismisses their points of view. Even though the children in my study were not alive during the 1980s or most of the 1990s, the changes that occurred during these years were etched into the social memory. It is impossible to overstate the importance of medicine in a context where people were left to die or the meaning of a TB or HIV diagnosis for families that have said goodbye to too many relatives. This history is part of children's communication about illness, the ways in which they involved themselves in care for themselves and their sick guardians, and their approaches to biomedicine, schooling, and much more.

I have shown throughout this book that children did not see themselves as passive, even within the most challenging contexts. Abby was one of the children who attended my first children's workshop, which focused on children's understandings of and contributions to health in George. Olivious, Emily, and I had practiced what we would say at the workshop, repeatedly. We wanted the children to know that we had organized the workshops specifically for the children to share their views, and that we did not intend to teach the children. As Olivious explained our purpose, the children sat stoically, hanging onto her words, but with few visible responses to what she was saying. Then she finally said: "In this workshop, you are the teachers. Today, we are learning from you." Abby jumped up, gasped loudly. With her hand over her mouth, she grinned widely. The rest of the children paused, smiled, and looked at one another.

Even this well-rehearsed, conscious effort to listen to children was challenging. Prior to the workshops, Emily expressed to me that she was anxious about the answers the children might come up with and how we should respond to them. To her, it made no sense to let the sessions play out with very little adult input. Emily is a counselor and wonderful teacher to her own children, nieces, nephews, and grandchildren. She held a keen awareness of the inequalities in access to knowledge the children faced. When the children started to give what in fact were wrong answers, I felt the same anxiety as Emily. Should I explain that ARVs were not a disease, as expressed by some children? What if the children learned incorrect information from my workshops and used what they learned to inform their understandings of illness and their interactions in their families? And what if they spread what they learned to their teachers, parents, and guardians, who would view me as giving misinformation or, alternatively, use what the children said to inform their own views? My acknowledged concern that the children would see me as a teacher obscured my own rather subconscious understanding of myself as a knowledge producer. In the end, we thanked the children for participating in the workshops and told them how much we had

learned from them. We framed the conclusion to emphasize the biomedically accurate information given to us by the children and downplayed answers that did not fit into our belief system.

What is important is the contradiction between how social science researchers may view adults' responses and children's responses differently and what this obscures in our analyses. For example, I at times heard adults say that HIV or AIDS comes from or is produced by TB. This, to me, was an astute observation that, in few words, tells us that HIV status was often unknown until a person faces a crisis illness such as TB. It also revealed the unidirectional flow of TB patients into HIV counseling and testing services and the close social and biomedical connections between TB and HIV. In other words, I did not see this phrase as particularly inaccurate according to biomedical understandings of HIV and TB, even though it was technically incorrect. In contrast, when children told me that ARV was a disease or that ARVs cured TB, I immediately wanted to correct them and I viewed the children as misinformed. A closer analysis of children's accounts of ARVs as disease, however, reveals many important things about the ways in which children learned about disease through medicine. It reveals one of many ways in which people in George maintained sociality and expressed care for one another through invoking medication rather than diagnosis. So often, as adults, we feel the need to teach children and prepare them for the future. We forget just how much they have to offer us in the present.

To view children as agents is to give credit where credit has long been due for children's in-depth knowledge of their social worlds, and also for their actions in shaping those worlds. Children, as I have shown throughout this book, do many things that we are not paying attention to because we do not expect or want children to do them and because our methodologies for viewing such problems exclude children. The frequency with which the children in my study ensured that adults received and took their TB medications is one case in point.

Yet, we must be careful how we envision children as health agents. In much public health work, children have been recruited to share information and encourage particular health behaviors. This, in my opinion, is not honoring children's agency. It is seeing them as megaphones to disseminate messages more widely than public health actors can typically reach. Such a view exemplifies an adult or elitist perspective—the intentions of children who participate in such interventions are left unexamined because they are expected to align with those of public health actors.⁶

To view children as health agents is to honor children's everyday efforts to maintain dignity and sustain lives within exceptionally difficult circumstances. This means seeing them as part of the social basis of health and exploring the entanglements of the social and biomedical as experienced, enacted, and lived by children.⁷ This is not to argue that we privilege children's actions and perspectives

within illness.⁸ It is to suggest the necessity of viewing children as people who act within social contexts and in relation to and relationship with other people in their families and communities.

BEYOND CHILDREN-AS-VICTIMS: TOWARD AN UNDERSTANDING OF ADULTS' HEALTH AGENCY

A move toward understanding children's health agency holds potential to open up new interpretations of the health agency of adults. Dominant discourses about HIV, TB, and other infectious diseases frequently focus on children as the innocent victims of the risky behaviors of adults: babies exposed to HIV through mother-to-child transmission, children orphaned when parents die of HIV-related infections, and children exposed to abuse and labor (Fassin 2013). This framing of children as passive victims of adult actions is related to the preoccupation with individual (adult) behavior change that has dominated global public health responses to infectious disease.⁹ When paired with the notion of children as victims vis-à-vis adult behavior, it not only exaggerates adult agency but also exacerbates the blaming of adults for children's circumstances, especially adults who fail to alter their behaviors in specific ways.

My discussion of disclosure to children offers an example of the necessity of a relational view of children's and adults' health agency. In recent years, public health practitioners and researchers have identified nondisclosure of HIV status to children as a behavior that adults might change to the benefit of their children and themselves. This suggestion holds many assumptions, two of which I cover below: first, that adult talk is unconstrained; and, second, that adults possess the power to give children agency or take it away. Both assumptions are problematic—the first because it exaggerates adults' agency and ignores the histories and lived experiences that have shaped such concealments and the second because it simultaneously denies children health agency and exaggerates their agency.

First, issues of power and social position shape what is said, who does the talking, and what remains unsaid. This clearly puts children at a structural disadvantage in terms of speaking and being told about HIV and TB. However, this does not mean that adult talk is unconstrained. As the guardians in my study showed, there were many constraints on their talk related to the extreme and uncertain conditions in which they lived and their desires to nurture their children within such conditions. Much work on disclosure of parental HIV to children has not adequately addressed these constraints. A lack of direct naming is often framed as cultural, with silence equated with cultural differences in talk about sex and death with children. While identifying that adults face challenges in speaking about HIV, such an analysis of silence as cultural overlooks

the historical, economic, and social factors that have compelled concealment. It conflates structural violence and cultural difference.¹⁰

Second, in the commentaries on disease disclosure, silence and disclosure are positioned in opposition, with silence implying passivity and a lack of communication and disclosure standing for action and communication. However, instead of being passively unaware, children held detailed knowledge of TB in the absence of a named disease. Children actively collected knowledge of disease through attention to small signs on their sick relatives' bodies and changes in their houses and households. Children, too, engaged in strategic concealments to show care to the sick. Their awareness and actions complicate the dichotomous thinking embedded in the research on disclosure, which has led to claims that adult disclosure will shift children from passive victims of HIV to active participants in shaping their own care and that of their family members. As I have shown, belief in this trajectory ignores the amount of care and communication accomplished in the absence of a named disease and also exaggerates children's abilities to change their situations when disease is named.

My analysis of disclosure raises a further point about adult behavior change interventions aimed at children's well-being. The conceptualization of disclosure as beneficial to children is grounded in a not-so-subtle assumption that outsiders know what type of communication is best for children. This is a privileged gaze that allows a willful ignorance of other forms of communication. I have suggested a different starting point, which identifies that communication and action can take many forms that may not be readily obvious to outsiders. This starting point allows us to investigate a range of other important issues, such as how people approach intergenerational care, maintain relationships, and attempt to secure resources within tremendous constraint. Investigating these aspects brings us closer to answering the bigger question that underpins almost all research and commentaries on disclosure to children, which is: How might the needs of sick adults and their young children be met to permit their continued relationships with one another?

WHAT WE MISS WHEN WE MISS CHILDREN

Many globally produced policies and programs explicitly target children. There are many others that target adults and overlook children almost entirely. Until 2013, when the World Health Organization declared childhood TB as one of its priority areas, children were rarely commented on in debates and discussion about TB treatment. And even still, the emphasis on childhood TB has not been on understanding children's experiences and social worlds, but on developing and providing technologies—diagnostic tools and specialized child-friendly TB treatment formulas. For this reason, I ask: What do we miss when we overlook children's experiences, actions, and perspectives?

Throughout this book, I have offered many reasons for why we cannot dismiss children. I reinforce these reasons with yet another example, journalist David Baron's (2010) account of healthcare rationing in Zambia, which was part of a Public Radio International series on healthcare rationing across four countries—South Africa, England, India, and Zambia. In Baron's report, he suggested that, in Zambia, healthcare is unintentionally rationed by implicit factors. The implicit factor he described in his report was the long queues at ARV clinics since the roll-out of no-cost treatment. This "rationing by queue" is of course no revelation to Zambians, who will readily divulge and complain about how long they have to wait to receive public healthcare. Baron went on to suggest that a variety of factors determined whether or not a person could wait. As he suggested: "This kind of implicit rationing may discourage certain categories of patients—for instance, those with young children or with full-time jobs."

As biomedical technologies to treat TB and manage HIV become increasingly available around the world and as pharmaceuticals become a main form of public health delivery, there has been an increasing recognition that relationships affect treatment seeking. Community support is now recognized as integral to the implementation of DOTS protocol, and a number of NGOs have assisted the government health centers in Zambia to make community DOT possible. But these programs of patient support come and go in George, with the ebb and flow of international funding. The TB treatment supporters I have known through the years are dedicated, but they are overworked and their work is frequently unremunerated. In other words, despite these efforts and changes, treatment adherence continues to be made possible (or not) through familial relationships. A series of social and economic differences structure the ways in which particular individuals are able to wait for medications and others are not, with age- and gender-related effects. Women carry out much work of collecting medicine, especially medicine for men. Other people, mostly women and children, make waiting in queues possible by providing childcare or household work.

The children I knew in George were integral to TB treatment adherence. They carried out household chores, sibling care, and care for the sick that made the collection of medication possible, raising questions about how children figure into such a system of healthcare rationing. Beyond medicine collection, the children expounded on the rules of treatment and reminded adults to take their medications. They knew where medicines were stored and when they needed to be taken. The reality that children were doing something to make treatment accessible and adherence possible is worthy of acknowledgment. While TB treatment is available, this does not mean that it is accessible. The institutional and medical infrastructure to ensure access is much better than it was in the past, but it is still severely lacking. Children help make up for the absences. It may not be

an exaggeration to say that the efforts of children to keep people on treatment optimally benefit the state and beyond.

TB treatment is a resource for children at a time when resources are especially scarce. As I have shown, there were so many things about TB that excluded children. Children were not often welcomed in TB Corners. They were sometimes moved out of households as families attempted to accommodate the needs of patients and avoid transmission to children. In most households I worked in, adults expressed a very real concern about spreading TB to their children. Clinical advice to avoid transmission and received wisdom, passed down from more stringent clinical rules in the previous decades, took away some things that affirmed children's relationships with their guardians, such as eating together and sleeping close. In such a context, the availability of pharmaceuticals and the ideals about patient support became vital resources for children, even though neither was intended for them. As I have shown, children repurposed these materials and rules, offering their own forms of patient support. In the process, they enacted claims to particular people and households and worked to transform their relationships and retain normative identities. There was much at stake, including the biological life of a beloved relative and the social life and future of the child.

REMAINING CLOSE

As my research in the households drew to a close, I worried about the effects of an abrupt departure on some of the children with whom I was especially close. I carried out a number of activities to thank them for spending time with me. The last set of children's workshops that I held celebrated children's participation in the study. I expressed my gratitude through words and small gifts. And on my final visits to each household, I gave cards to the households, with enclosed pieces of paper that described some of my research findings. I did not wish to be the researcher who collected her data and never returned, though life circumstances have led me closer to these fears than I feel comfortable admitting. My soonest return would not be until 2014.

Back in 2008, on what I assumed would be my final visit to Irene's household, I gave Irene and her brother a stack of paper. They had drawn through reams during the year I had been visiting them and, while I knew I should look beyond the aesthetic appeal of their drawings, I still assumed that they enjoyed drawing as a personal endeavor. Two weeks later, I passed by their home unexpectedly to give them some books I had received from a friend as a donation to the children in my study. Irene was clearly surprised to see me, saying: "I didn't draw anything for you because I didn't think you were coming back." At that moment, only after the research had ended, I realized that her drawings served as a means

to cultivate a relationship with me, to be closer to me than she might have been without the drawings.

If we do not question the ways in which children engage us as researchers, we will never fully comprehend what they are telling and showing us. In my research, children's uses of my methods fit within their overall strategies to sustain relationships. The drawings children gave me as gifts and drew as assignments induced me to return to their homes and strengthened our connections to one another. Even if they were skeptical of the topics I suggested, most children completed the assignments, demonstrating that help is what nurtures relationships. It is with this understanding that I view Abby's final drawing to Olivious and me with mixed feelings. The drawing was of colorful flowers, hearts, and curvy designs; the word "love" embedded into the shapes. At the top of the page, she wrote in large letters "I Love You Olivia and Jean." In smaller lettering on the bottom she wrote, "This is love for you."

The love Abby offered to Olivious and me reminds me that we, too, became enmeshed in social relations of care, if only for a brief moment in time. Expressing love was how the children held onto relationships that mattered, when they were at their most fragile. Putting love into words sustained relationships and compelled people to be closer to each other, both physically and emotionally. When attempts to maintain physical proximity failed, expressions of love became attempts to retain closeness, to remember people once they were gone.¹¹ I am reminded of eleven-year-old Eliya, who addressed his father, as he whispered softly into my recorder, "Father, don't worry. I will take care of my mother and always love you. I will never forget you because I know you are happy, wherever you are. You were very kind. May you rest in peace."